



MEDICATION DOSES: EPI 1 mg q3–5 min | AMIO 300 mg → 150 mg | LIDO 1–1.5 then 0.5–0.75 mg/kg | ADEN 6 mg → 12 mg (rapid IV push) | ATRO 1 mg q3–5 min (max 3 mg) | Ca gluconate 10% 1–2 g/10–20 mL (or CaCl₂ 1 g) | NaHCO₃ 50 mEq/50 mL
TREATMENT: CPR — Absent : 0 / Present : ✓ (x2-min rounds) | SHOCK — Biphasic : 120–200 J / Monophasic : 360 J | TIME in military format

[illegible]

Recording grid · Event conclusion · Personnel on duty

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TIME MILITARY	BP	HR	RHYTHM	O ₂ SAT	PULSE ABS 0 / ✓	CPR ✓ /2MIN	SHOCK JOULES	EPI 1MG	AMIO 300/150	LIDO MG/KG	ADEN 6/12	ATRO 1MG	CA GLUC 1–2G	NAHCO ₃ 50MEQ	COMMENTS BVM · INTUBATION · IV FLUIDS · PACING · CARDIOVERSION · LINES · LABS · ETC

CODE BLUE EVENT CONCLUSION

Time event ended: _____

Patient outcome: ☐ ROSC achieved ☐ Expired / efforts terminated

Reason resuscitation ended: ☐ ROSC ☐ No benefit to continue

Family notification: ☐ At bedside ☐ By phone ☐ Unable to reach ☐ No family ☐ Pending

Name of person notified: _____

Relationship to patient: _____

PERSONNEL ON DUTY

_____ PRIMARY NURSE	_____ CHARGE NURSE	_____ RECORDER
_____ CPR STAFF	_____ MEDICATION STAFF	_____ RESPIRATORY THERAPIST
_____ RESIDENT ON DUTY (ROD)	_____ PHYSICIAN	_____ ON-CALL NEPHROLOGIST

START CPR — push hard & fast 100–120/min, depth 5–6 cm, full recoil · give O₂ · attach monitor / defibrillator

DIALYSIS ACTIONS (in parallel): STOP ultrafiltration · saline bolus 200–500 mL · return blood or clamp & disconnect — KEEP vascular access for drugs · think HYPERKALEMIA early

Rhythm shockable?

SHOCKABLE — VF / PULSELESS VT

SHOCK · biphasic 120–200 J (device-specific) · monophasic 360 J

CPR 2 min · obtain IV / IO access

Rhythm shockable? → **SHOCK**

CPR 2 min · **Epinephrine 1 mg** IV/IO q3–5 min · consider advanced airway + capnography

Rhythm shockable? → **SHOCK**

CPR 2 min · **Amiodarone 300 mg** (then 150 mg) or Lidocaine 1–1.5 mg/kg · treat reversible causes

ROSC? → post-cardiac-arrest care. If not, continue 2-min cycles.

Reversible causes — H's: **HIGH-YIELD** Hyper-/hypokalemia · Hypovolemia (over-UF → saline) · Hydrogen ion / acidosis · Hypoxia · Hypothermia · Hypocalcemia / hypomagnesemia

If hyperkalemia suspected (missed sessions, Monday gap): empiric **Calcium gluconate 10% 1–2 g IV** (or CaCl₂ 1 g) → insulin 10 U + D50 25 g → salbutamol neb → ± bicarbonate → **emergency dialysis, low-K bath**. Calcium and bicarbonate must not share a line.

NON-SHOCKABLE — ASYSTOLE / PEA

CPR 2 min · **Epinephrine 1 mg** IV/IO as soon as possible, q3–5 min · IV/IO access · advanced airway + capnography

Rhythm shockable? → if YES, move to the shockable lane

CPR 2 min · treat reversible causes (see below)

ROSC? → post-cardiac-arrest care. If asystole persists despite treating reversible causes, consider termination per team decision.

Reversible causes — T's: Thrombosis (MI / PE) · **AIR EMBOLISM** clamp venous line, left-lateral Trendelenburg · Tamponade · Tension pneumothorax · Toxins

